CJC-1295 NODAC + Ipamorelin Available for Research Use Only

150–200 mcg each, SC once or twice daily; fasted, 8–16 weeks on, 4–6 weeks off

CJC-1295 (no DAC) + ipamorelin is an experimental GH-secretagogue combination that likely enhances GH release, with plausible applications in body composition, sleep, and recovery. [2–4,6,8,14,19,20] CJC-1295 (no DAC), commonly called modified GRF(1–29), is a GHRH analog distinct from the long-acting, albumin-binding DAC compound. [4,22,23] Ipamorelin is a ghrelin-receptor (GHSR-1a) agonist that stimulated GH release in human volunteers. [2,6] Complementary receptor signaling provides a plausible basis for synergy and downstream IGF-1 elevation. [3,8] Animal selectivity findings support plausible sparing of cortisol and prolactin responses in humans. [2]

Safety depends on the endpoint, preparation, and exposure duration; the combination requires separate assessment from its individual components. [7,15,23,24]

💉 Plausibly safe

⭐ ⭐ Likely enhancement of GH release, with plausible benefits for body composition and recovery. [2–4,6,8,14,15]

C Plausible tumor-growth concern through GH/IGF-1 signaling. Avoid use with active cancer; prior cancer or elevated cancer risk warrants individualized assessment. [17,22]

🔬 Testing encouraged — monitor IGF-1 against the age-adjusted reference range; levels above that range warrant reassessment of use and dose. [16,22]

Disclaimer: These notes are not authoritative and should not be interpreted as definitive scientific guidance. They are summaries, compilations and plausible extrapolations drawn from the most relevant research I was able to locate, and may omit nuances, conflicting data, or emerging evidence. The material reflects an attempt to organize and understand complex topics, not to provide clinical recommendations or expert interpretation.

Safety assessment draws on separately administered ipamorelin and the DAC form of CJC-1295. In a postoperative trial with 114 patients in the safety analysis, IV ipamorelin for up to 7 days was described by the investigators as well tolerated; treatment-emergent adverse events occurred in 87.5% versus 94.8% with placebo. [7] The surgical population, IV route, and short exposure limit extrapolation to repeated SC combination use. [7,24]

CJC-1295 (no DAC) + ipamorelin likely enhances GH release, with downstream elevation of IGF-1. This inference draws on human ipamorelin findings, GHRH-analog pharmacology, and complementary receptor signaling. [2–4,6,8] The sustained human GH/IGF-1 responses reported for CJC-1295 were obtained with the DAC formulation but plausibly apply to repeated NODAC administration. [1,5]

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Body-composition benefits remain plausible from GH/IGF-1 biology. [15] Ipamorelin increased adiposity in mice, including through GH-independent effects, which complicates the fat-loss hypothesis for this combination. [11]

Dosage & Patterns

Human IV ipamorelin research demonstrated dose-dependent GH responses. [6]

The speculative experimental pattern discussed in peptide communities and wellness spas is SC administration once or twice daily, while fasted, before bedtime or immediately after waking. [21,22]

- **150–200 mcg of each peptide per administration**

Reported empirical schedules include 8–12 or 12–16 weeks of use, generally followed by 4–6 weeks off. [22]

- **8–12 weeks on with 4 weeks off, or 12–16 weeks on with 4–6 weeks off.**

Preservation of response through planned breaks remains speculative. [22]

Benefits

- **Plausible improvement in sleep quality through GHRH-related sleep mechanisms**. GHRH improved sleep in a study of men but impaired sleep in a study of young women. [19,20] Easier sleep onset and vivid dreaming remain speculative outcomes for this combination.
- **Plausible support for recovery from training through GH/IGF-1-related tissue-repair signaling**. [14] Less next-day soreness and a faster return to baseline performance after heavy sessions are speculative functional outcomes of that mechanism.
- **Plausible support for connective-tissue repair**, informed by increased tendon collagen synthesis during a 14-day human GH study. [14] A qualitative study of female CJC-1295 users recorded anecdotal joint-comfort improvements in a setting of varied formulations and frequent use of other substances. [21] Tendon resilience and faster soft-tissue healing remain plausible translational outcomes.
- **Improved workout quality and muscle fullness are plausible** experiential outcomes of GH-axis stimulation and changes in tissue water or body composition. [15,22] The cited human GH study increased tendon collagen synthesis without increasing myofibrillar protein synthesis. [14]
- **Plausible support for body-composition changes through GH/IGF-1-related signaling**. [15] The female-user netnography recorded anecdotal perceptions of looking slimmer, amid multiple co-exposures. [21] Increased adiposity in ipamorelin-treated mice is a counterpoint to an uncomplicated fat-loss expectation. [11]
- **Calmer evenings, greater stress tolerance, and emotional resilience remain speculative** extensions to this combination. A qualitative study recorded anecdotal improvements in mood and outlook among female CJC-1295 users. [21]

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- **Plausible support for skin quality through GH/IGF-1-related collagen and tissue-remodeling signaling.** [14] Female CJC-1295 users described skin changes anecdotally. [21] Improved hydration, elasticity, and tone are speculative specific outcomes.
- **Greater training readiness and reduced cumulative fatigue are speculative f**unctional extensions of recovery-related GH/IGF-1 signaling. [14]

Applications

- Recovery from training is a proposed use informed by tissue-repair mechanisms. [14,21]
- Body recomposition is a reported community goal. [21,22]
- Reported use aimed at age-related declines in GH secretion (age-related decline is distinct from clinically diagnosed GH deficiency). [16,21,22]

Cautions

- Active malignancy: avoid use because of plausible tumor-supportive GH/IGF-1 signaling. [17,22]
- Uncontrolled diabetes: GH-related worsening of glycemic control is a material concern. [15,16,24]

Side Effects

- Potential water retention and joint discomfort. [15,22]
- Appetite may increase: ipamorelin increased food intake in animal studies, and CJC-1295 users described increased hunger in the qualitative study. [11,13,21] GH stimulation can impair insulin sensitivity and plausibly increase dysglycemia risk. [15,24]

Long Term Safety Signals

Endocrine and metabolic safety: GH-related glucose intolerance is a plausible risk. [15,16] In the postoperative IV ipamorelin trial, FDA reported hyperglycemia at discharge in 14.3% versus 8.6% with placebo and hypokalemia in 12.5% versus 3.4%. These differences arose in a small surgical study and do not establish causation or predict rates with SC combination use. [7,24]
Fluid retention and musculoskeletal safety: Edema and joint discomfort are recognized effects of GH exposure and plausible effects of this combination. [15] Female CJC-1295 users also reported bloating and joint pain during prolonged use; those uncontrolled reports involved variable products and other exposures. [21]
Cardiovascular and neurological safety: FDA noted two deaths in the postoperative ipamorelin group versus none with placebo, with uncertain causal attribution. Insomnia occurred in 10.7% versus 5.2%. [24] DAC-form CJC-1295 studies reported flushing, headache, and transient cardiovascular effects; that formulation has a different exposure profile. [23] These findings support monitoring rather than a confident long-term low-risk inference for the combination.
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Hepatic and renal safety: Short DAC trials showed no consistent liver-laboratory changes. [23] This observation is specific to the studied preparation and duration. Long-term hepatic and renal tolerability of the SC combination cannot yet be assigned a confident low-risk classification from those findings.

Reproductive and proliferative safety: Reproductive and developmental risks remain insufficiently characterized; ghrelin-receptor biology raises a plausible concern about pregnancy-related outcomes. [24] Tumor-growth mechanisms and clinical GH-replacement experience are assessed separately below. Rare or delayed outcomes require a different inference from common short-term adverse effects. [17]

Drivers of Diminished Response and Mitigation Strategies

Receptor desensitization plausibly contributes to diminished response during repeated stimulation. [3] After 15 days of ipamorelin in female rats, the response to ipamorelin was modestly reduced, while the GHRH response was preserved. [9] A separate 21-day rat study found dynamic changes in somatotroph GH content with retained responses in cell culture. [12] These findings suggest context-dependent adaptation rather than uniform loss of responsiveness. Immunogenicity is a potential concern. [23,24] IGF-1-mediated negative feedback is established physiology and plausibly limits the GH response. [3,16]

Intermittent dosing and planned breaks are empirical practices whose ability to preserve response remains speculative. Periodic IGF-1 monitoring can help assess biochemical response and guide reassessment. [16,22]

CJC-1295 NODAC + Ipamorelin and Cancer Risk

GH/IGF-1 signaling provides a plausible tumor-growth concern, particularly with active or residual malignancy. The concern extends to interventions that chronically raise GH and IGF-1: these pathways support proliferation and resistance to apoptosis in susceptible tumor systems. [17,22]

Acromegaly is associated with increased incidence of certain cancers, particularly colorectal and thyroid cancers. These observational findings **support concern about prolonged GH/IGF-1 excess but do not distinguish tumor initiation from promotion** or quantify risk from this combination. [18] Endogenous GH stimulation plausibly retains physiological feedback regulation. [3] Its comparative cancer risk depends on the resulting exposure and clinical context; preserved feedback alone is not a measure of oncological safety. [17,22]

Mechanistically, a microscopic tumor or residual disease could gain a growth advantage from increased GH/IGF-1 signaling. Experimental tumor biology supports this concern in susceptible breast, prostate, colorectal, and sarcoma systems; susceptibility differs across tumor types and molecular contexts. [17]

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Avoid use with active cancer. Prior cancer, recent treatment, or strong familial or genetic risk warrants individualized specialist assessment rather than a single rule for all patients. [17,22]

The magnitude and duration of GH/IGF-1 elevation plausibly influence tumor-growth signaling. Individual absolute cancer risk from this combination remains unquantified. [17,22]

Biological Mechanism

CJC-1295 (no DAC) acts through the GHRH receptor, while ipamorelin acts through the ghrelin receptor (GHSR-1a). [2–4] The combination likely enhances GH release, with plausible synergy through complementary receptor signaling and plausible downstream elevation of IGF-1. [3,6,8] Limited cortisol and prolactin responses in animal experiments support plausible endocrine selectivity in humans. [2]

Conclusions

CJC-1295 (no DAC) + ipamorelin likely enhances GH release, with plausible benefits for sleep, recovery, connective-tissue support, and body composition. [2–4,6,8,14,15,19,20] Animal adiposity findings and unchanged myofibrillar protein synthesis in the cited human GH study temper expectations for fat loss and muscle-fiber anabolism. [11,14] Fasting, bedtime administration, and intermittent schedules are empirical practices. [21,22] Safety should be assessed separately for glycemic control, fluid retention, immune reactions, and unresolved organ-specific or delayed outcomes. [15–17,23,24] Avoid use with active malignancy; prior cancer or elevated cancer risk warrants individualized specialist assessment. [17] Monitor IGF-1 against the age-adjusted reference range, with reassessment if levels exceed that range. [16,22]

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